Treatment scope · National Cancer Control Programme (NCCP) 00215 Lung context only. Confirm whether the platinum pathway or EGFR/erlotinib pathway applies. This guide does not supply the partner dose or replace its safety assessment.

NCCP 00215 · VERSION 7

Bevacizumab 15 mg/kg · lung treatment component

Bevacizumab 15 mg/kg is infused on Day 1 every 21 days. With platinum chemotherapy, the combined phase lasts up to six cycles, then bevacizumab may continue alone. With erlotinib, the combination continues while helpful and tolerated. Progression or unacceptable toxicity can stop either plan earlier; no fixed overall lung-treatment duration is promised.

Make treatment fit into daily life

The usual bevacizumab reference dose is 15 mg/kg intravenously every 21 days. The first infusion is over 90 minutes; if tolerated, the second can be over 60 minutes and later infusions over 30 minutes. The team checks the actual prescription, blood pressure, urine and other tests before treatment.

Infusion visits

The usual bevacizumab reference dose is 15 mg/kg intravenously every 21 days. The first infusion is over 90 minutes; if tolerated, the second can be over 60 minutes and later infusions over 30 minutes. The team checks the actual prescription, blood pressure, urine and other tests before treatment.

Checks and samples

Allow time for blood tests, a urine sample and blood-pressure checks. A 24-hour urine collection may be needed between visits; confirm when the team needs the result.

Partner treatment

The platinum chemotherapy schedule or daily erlotinib plan is separate. Bring the complete written plan. After up to six platinum cycles, confirm whether bevacizumab alone continues; this phase change does not apply automatically to erlotinib.

Access and procedures

Bevacizumab needs an intravenous cannula or existing suitable central line; the team decides access. Tell the team about surgery, dental work or wounds before booking procedures.

Support contacts

Keep the daytime and out-of-hours oncology numbers available. If a visit or test is missed, contact the team for a revised plan rather than rearranging treatment yourself.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00215 v7 · official regimen
- EMA Avastin product information · current snapshot
- Sandler et al., NEJM 2006: ECOG 4599 carboplatin/paclitaxel with or without bevacizumab
- Yamamoto et al., Lung Cancer 2021: JO25567 updated PFS and final OS
- NCCP antiemetic classification v6

Source reconciled; independent clinical, pharmacy and patient/PPI review pending